

GLP-1 WEIGHT LOSS MEDICATIONS — CLINICAL REFERENCE

SEMAGLUTIDE · TIRZEPATIDE · DOSING TITRATION · SIDE EFFECTS · LABS · DRUG INTERACTIONS · PATIENT MANAGEMENT

CHEAT SHEET

NO PRIOR AUTHORIZATION

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GLP-1 PRODUCT COMPARISON

Drug	Class	Brand	Route	Dosing Freq	FDA Indication
Semaglutide	GLP-1 RA	Ozempic/Wegovy	SubQ	Weekly	T2DM / Obesity
Tirzepatide	GLP-1 + GIP	Mounjaro/Zepbound	SubQ	Weekly	T2DM / Obesity
Liraglutide	GLP-1 RA	Victoza/Saxenda	SubQ	Daily	T2DM / Obesity
Dulaglutide	GLP-1 RA	Trulicity	SubQ	Weekly	T2DM only
Exenatide	GLP-1 RA	Byetta/Bydureon	SubQ	BID/Weekly	T2DM only

NOTE Tirzepatide dual agonist (GLP-1 + GIP) produces greater weight loss than semaglutide in head-to-head trials. Average: 20–22% body weight vs 15% semaglutide.

SEMAGLUTIDE TITRATION PROTOCOL

Weeks	Dose (SubQ weekly)	Notes
Weeks 1–4	0.25 mg	Initiation dose — not therapeutic
Weeks 5–8	0.5 mg	Increase if tolerated
Weeks 9–12	1.0 mg	Many patients respond here
Weeks 13–16	1.7 mg	Wegovy dosing step
Week 17+	2.4 mg	Max Wegovy dose — maintenance

SLOW If GI side effects are significant, hold at current dose for an extra 4 weeks before advancing. Never rush titration — patient retention depends on tolerability.

TIRZEPATIDE TITRATION PROTOCOL

Weeks	Dose (SubQ weekly)	Notes
Weeks 1–4	2.5 mg	Initiation dose
Weeks 5–8	5.0 mg	First therapeutic dose
Weeks 9–12	7.5 mg	Increase if tolerated
Weeks 13–16	10 mg	Most patients respond here
Weeks 17–20	12.5 mg	Continue titrating
Week 21+	15 mg	Max dose — maintenance

COMPOUNDED GLP-1 — REGULATORY STATUS

STATUS FDA declared **semaglutide shortage resolved (2024)** — compounded semaglutide 503B outsourcing facilities faced discontinuation orders. Status changes frequently — **verify current FDA guidance before prescribing.**

503A 503A compounding pharmacies (patient-specific): permitted for individualized patient prescriptions with documented clinical need. Physician oversight required.

503B 503B outsourcing: bulk compounding — regulatory scrutiny high in 2024–2025. Verify pharmacy is FDA-registered and compliant.

! **Never compound tirzepatide base** — not on FDA shortage list. Tirzepatide salts (acetate, citrate) — legal gray area, avoid until clear guidance issued.

CONTRAINDICATIONS

ABS **Absolute:** Personal/family history of **medullary thyroid carcinoma (MTC)** or Multiple Endocrine Neoplasia type 2 (MEN2). Hypersensitivity to product.

ABS **Pregnancy** — discontinue at least 2 months before planned pregnancy. Not studied in pregnancy. Teratogenic risk unknown.

REL **History of pancreatitis** — use with caution. Monitor amylase/lipase if symptoms arise. Contraindicated if active pancreatitis.

REL **Severe GI disease** — gastroparesis, IBD, prior GI surgery. GLP-1 slows gastric emptying — may worsen existing conditions.

REL **Diabetic retinopathy** — rapid glucose reduction can worsen retinopathy. Ophthalmology follow-up recommended.

REL **Renal impairment** — use caution. Semaglutide not renally cleared but GI side effects can cause dehydration + AKI. Monitor.

DRUG INTERACTIONS

Drug/Class	Interaction	Management
Insulin	↑ Hypoglycemia risk	Reduce insulin dose. Monitor BG closely.
Sulfonylureas	↑ Hypoglycemia risk	Reduce SU dose. Monitor.
Oral medications	Delayed absorption	Time-sensitive meds (thyroid, OCP) — take 1hr before injection.
Warfarin	↑ INR possible	Monitor INR more frequently when initiating.
Alcohol	↑ Nausea, hypoglycemia	Limit alcohol. Educate patient.

REQUIRED LABS

Lab	Timing	Why
HbA1c	Baseline + q3mo	Glycemic monitoring
Fasting glucose	Baseline + q3mo	Hypoglycemia risk
CMP (BMP)	Baseline + q3mo	Renal function, electrolytes
Lipid panel	Baseline + q6mo	Cardiovascular risk
TSH	Baseline	Thyroid cancer screening
Calcitonin	Baseline	MTC screening (some protocols)
LFTs	Baseline	Liver function baseline
UA + microalbumin	Baseline + annual	Renal protection monitoring
Weight + BMI	Every visit	Efficacy tracking
Blood pressure	Every visit	Cardiovascular monitoring

SIDE EFFECT MANAGEMENT

NAUSEA **Most common (40–50%)**. Peaks weeks 1–4 of each dose increase. Management: small frequent meals, avoid fatty/spicy food, ginger supplements, Zofran 4mg PRN, domperidone (off-label). **Most resolves within 2–4 weeks.**

VOMITING Persistent vomiting — hold dose escalation. Consider antiemetic prescription. Ensure hydration. If severe — delay dose, reduce to prior tolerated dose.

DIARRHEA Common especially tirzepatide. Loperamide PRN. Increase fiber slowly. Ensure adequate hydration. Usually self-limiting within 4–6 weeks.

CONSTIPATION GLP-1 slows GI motility. Increase water + fiber. MiraLax PRN. Encourage movement. Common at higher doses.

FATIGUE Common first 4–8 weeks. Ensure adequate caloric intake — minimum 1200 kcal. Prioritize protein. Usually self-resolving.

PANCREATITIS **Severe abdominal pain radiating to back, nausea/vomiting.** HOLD medication. Check amylase/lipase. ER referral if suspected. Do not rechallenge if pancreatitis confirmed.

GALLBLADDER Rapid weight loss increases cholelithiasis risk. Counsel patients. RUQ ultrasound if biliary symptoms develop.

NUTRITION PROTOCOL

PROTEIN **Priority #1: Minimum 1g protein per pound of goal body weight** daily. GLP-1 causes muscle loss if protein inadequate. Lean meats, Greek yogurt, eggs, protein shakes.

CALORIES GLP-1 reduces appetite significantly. Ensure minimum **1200 kcal/day (women) / 1500 kcal/day (men)**. Too-low calories = muscle wasting + nutrient deficiency.

HYDRATION **Minimum 64–80 oz water daily.** Dehydration risk increases with nausea/vomiting/diarrhea. Electrolytes if significant GI losses.

AVOID Fatty, fried, spicy foods worsen nausea. Alcohol + GLP-1 = increased hypoglycemia + worsened GI effects.

EXERCISE Resistance training essential to preserve muscle mass during weight loss. Minimum 2x/week strength training. Cardio adjunctive.

MONTH-BY-MONTH EXPECTATIONS

Month 1 → **GI adjustment** →

Month 2: Loss begins →

Month 3: Visible change →

Month 6: 10–15% lost

M1 **Month 1:** Focus on tolerating medication. Weight loss minimal — 1–3 lbs. GI side effects peak. Do not assess efficacy yet.

M2 **Month 2:** Appetite suppression more noticeable. 2–4 lbs/month average. Energy stabilizing. Sleep improving.

M3 **Month 3:** Visible results. Average 5–10% body weight lost. Patients start referring friends. Rebook window — schedule month 4 check-in.

INJECTION TECHNIQUE

SITES **Rotate weekly:** Abdomen (2 inches from navel), anterior thigh, upper arm (outer). Document rotation. Never inject same site consecutively.

TECHNIQUE Pinch 1 inch of skin. 90° angle (45° if thin). Insert full needle length. Inject slowly over 5–10 seconds. Hold 5–10 seconds after. Remove. Do not rub.

TIMING Same day each week. Time of day does not matter — patient preference. Can take with or without food. With food if nausea is significant.

STORAGE **Unopened:** Refrigerate 36–46°F. Do not freeze. Once opened: refrigerate or room temp up to 56 days (semaglutide), 30 days (tirzepatide). Protect from light.

MISSED **Missed dose:** If within 5 days — inject as soon as remembered. If more than 5 days — skip and resume next scheduled dose. Never double dose.

WHEN TO DISCONTINUE

STOP Confirmed pancreatitis. MTC diagnosed. Severe allergic reaction. Pregnancy confirmed — discontinue immediately, refer to OB.

STOP Persistent vomiting/inability to eat for 48+ hours — hold until resolved. Reassess dose reduction vs discontinuation.

CONSIDER No weight loss after 16 weeks at therapeutic dose — reassess adherence, nutrition, dose. Consider alternative medication or evaluation for secondary causes.

REBOUND Weight regain is expected on discontinuation — counsel every patient before starting. This is chronic disease management, not a temporary treatment.

HIGH-YIELD CLINICAL PEARLS

★ **Slow titration = patient retention.** The #1 reason patients stop is GI intolerance from rushing the dose.

★ **Set expectations at month 1.** Month 1 is about tolerating the drug, not losing weight. Patients who know this stay on the program.

★ **Protein is non-negotiable.** Muscle loss is real. Patients who don't hit protein targets will lose muscle not just fat.

★ **Tirzepatide is more effective** but also more expensive and more GI side effects at initiation. Start semaglutide if cost is concern.

★ **Verify compounding pharmacy regulatory status** every quarter. The landscape changes — protect your license and your patients.

★ **MTC black box warning** is based on rodent studies. Human risk unclear but contraindication stands. Always screen family history.

★ **Oral medications timing matters.** Thyroid medication, oral contraceptives — take 1 hour before GLP-1 injection day to avoid delayed absorption.

QUALIFYING CRITERIA (FDA/INSURANCE)

Criteria	Threshold
BMI for obesity treatment	BMI ≥30 kg/m ²
BMI + comorbidity	BMI ≥27 + T2DM, HTN, dyslipidemia, OSA, or CVD
Prior weight loss attempts	Documented lifestyle intervention
T2DM indication (Ozempic)	HbA1c ≥7% on lifestyle alone

